Address : NEW No 86, OLD No 28, PLOT No 30, MAHADEVAN STREET, WEST MAMBALAM, CHENNAI – 600033 TAMILNADU.

SID No : **22000335**
Branch : **WEST MAMBALAM**
Mr. PRASANTH.S
Age / Sex : 31 Y / Male
Ref. By : **MEDIBUDDY-RETAIL**

Patient ID : 2201425655

Collected Date : 01/04/2026 / 15:44
Received Date : 01/04/2026 / 16:07
Reported Date : 01/04/2026 / 17:54

Final Test Report

Test Name / Specimen	Result	Units	Reference Range / Method
HAEMATOLOGY			
CBC-13			
Total WBC count EDTA BLOOD	7600	cells/cumm	4000 - 10000 Coulter Principle MC-7911
DIFFERENTIAL COUNT.			
NEUTROPHILS EDTA BLOOD	53.07	%	40-70 VCSn Technology MC-7911
LYMPHOCYTES EDTA BLOOD	36.80	%	20-45 VCSn Technology MC-7911
EOSINOPHILS EDTA BLOOD	3.44	%	1-7 VCSn Technology MC-7911
MONOCYTES EDTA BLOOD	6.16	%	2 - 7 VCSn Technology MC-7911
BASOPHILS EDTA BLOOD	0.53	%	0-1 VCSn Technology MC-7911
Haemoglobin EDTA BLOOD	15.8	g/dL	13.0 - 17.0 Photometry MC-7911
PCV EDTA BLOOD	46.7	%	40.0 - 54.0 Calculated MC-7911
Red Blood Cell (RBC) Count EDTA BLOOD	**5.55** ↑	million/cumm	4.50 - 5.50 Coulter Principle MC-7911
MCV EDTA BLOOD	84.1	fl	83.0 - 101.0 Derived from RBC Histogram MC-7911
MCH EDTA BLOOD	28.5	pg	27.0 - 32.0 Calculated MC-7911
MCHC EDTA BLOOD	33.8	g/dL	31.5 - 34.5 Calculated MC-7911
Platelet count EDTA BLOOD	296000	cells/cumm	150000 - 410000 Coulter Principle MC-7911

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Dr.K.S.Karthikeyan MD (Path)

Consultant Pathologist

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BIOCHEMISTRY			
Glucose, Fasting Fluoride	83.2	mg/dL	Healthy Adult or children : less than 100 Pre diabetic : 100 – 125 Diabetic : 126 or above (ADA 2019) Enzymatic : hexokinase

MC-7911

Dr.G.Kavitha MD.,
Consultant Biochemist

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Test Name / Specimen	Result	Units	Reference Range / Method
BIOCHEMISTRY			
Lipid Profile			
Cholesterol, Total Serum	**237.1** ↑	mg/dL	Adult (NCEP ATP-III) Desirable : <200 Borderline high : 200 - 239 High : >239 Enzymatic : CHOD-PAP MC-7911
Note: Above Biological interval is based on 9 to 12 hours fasting.			
Triglycerides Serum	148.60	mg/dL	Adult (NCEP ATP-III) Normal : <150 High : 150 - 199 Hypertriclyceridemic: 200 - 499 Very High : >499 Enzymatic Colorimetric : GPO-PAP MC-7911
Note: Above Biological interval is based on 9 to 12 hours fasting			
Cholesterol, HDL Serum	40.6	mg/dL	Adult (NCEP ATP-III) Low : < 40 High : >=60 Enzymatic-Colorimetric MC-7911
Cholesterol, LDL Serum	**166.8** ↑	mg/dL	Optimal : <100 Near or above optimal : 100 - 129 Borderline high : 130 - 159 High : 160 - 189 Very high : >190. Calculation MC-7911
Cholesterol, VLDL Serum	29.7	mg/dL	Less than 30 (NCEP ATP-III) Calculation MC-7911
Cholesterol/HDL Ratio Serum	**5.8** ↑	Ratio	Castelli's Risk Index -I Ideal : <3.5 Good: 3.5-5.0 High: >=5 Calculation MC-7911
LDL/HDL Ratio Serum	4.1	Ratio	Castelli's Risk Index -II Ideal : <2.0 Good: 2.0-5.0 High: >=5 Calculation MC-7911

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Test Name / Specimen	Result	Units	Reference Range / Method
Non - HDL Cholesterol Serum	**196.5** ↑	mg/dL	Adult (NCEP ATP-III) Optimal : <130 Near or above optimal : 130-159 Borderline high : 160-189 High : 190-219 Very high : >220 Calculation MC-7911
HDL/LDL Ratio Serum	0.2	Ratio	Optimal : >0.4 Moderate: 0.3-0.4 High : <0.3 Calculation MC-7911

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BIOCHEMISTRY			
LIVER FUNCTION TEST WITH GGT			
Bilirubin, Total Serum	0.81	mg/dL	0.30 - 1.20 Colorimetric : Diazo MC-7911
Bilirubin, Direct Serum	0.26	mg/dL	0 - 0.2 Colorimetric : Diazo MC-7911
Bilirubin, Indirect Serum	0.55	mg/dL	0.1 - 1.0 Calculated MC-7911
Aspartate aminotransferase (AST/SGOT) Serum	30.30	U/L	Upto 40 IFCC without P5P MC-7911
Alanine aminotransferase (ALT/SGPT) Serum	**46.00** ↑	U/L	Upto 41 IFCC without P5P MC-7911
Alkaline phosphatase Serum	89.0	U/L	40.0 - 129.0 Colorimetric : p-Nitrophenyl Phosphate-AMP Buffer MC-7911
Gamma Glutamyl-Transferase (GGT) Serum	61.0	U/L	10.0 - 71.0 Colorimetric : IFCC MC-7911
Total Protein. Serum	8.00	g/dL	6.40 - 8.30 Colorimetric-Biuret MC-7911
Albumin. Serum	4.83	g/dL	3.50 - 5.20 Colorimetric: Bromocresol Green MC-7911
Globulin. Serum	3.17	g/dL	2.0-3.9 Calculated MC-7911
Albumin/Globulin Serum	1.5	Ratio	Calculated MC-7911
Kidney Function Profile			
BUN Serum	**5.1** ↓	mg/dL	6 - 20 Kinetic Urease/GLDH MC-7911
Creatinine. Serum	0.99	mg/dL	0.90 - 1.30 Jaffe MC-7911
Uric Acid. Serum	5.70	mg/dL	3.50 - 7.20 Enzymatic-Colorimetric MC-7911

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Test Name / Specimen	Result	Units	Reference Range / Method
Est. Glomerular Filtration Rate Serum	88.0		>=18 years : >=60 ml/min/1.73m^2 MDRD calculation MC-7911

Estimated Glomerular Filtration Rate (eGFR) test is useful

- Diagnosing and monitoring treatment of acute and chronic kidney diseases (CKD)
- Adjusting dosage of renally excreted medications
- Monitoring kidney transplant recipients
- Estimating glomerular filtration rate for people with chronic kidney disease (CKD) and those with risk factors for CKD (diabetes, hypertension, cardiovascular disease, and family history of kidney disease)

eGFR calculated using the Modification of Diet in Renal Disease (MDRD) Study equation provides a more clinically useful measure of kidney function than serum creatinine alone. This equation takes into account several factors that impact creatinine production, including age, gender, race and Cr method traceable to IDMS. In general, quantification of eGFR values below 60 mL/min/1.73 m^2 has more clinical implications for classification of kidney function than values above this level (NIDDK).

Limitations:

Creatinine-based estimating equations are not recommended for use with:

- The equation has been validated only for above the age of 18 years with impaired kidney function.
- Individuals with unstable creatinine concentrations like pregnant women; patients with serious co-morbid conditions; and hospitalized patients, particularly those with acute renal failure. Creatinine-based estimating equations should be used only for patients with stable creatinine concentrations.
- Persons with extremes in muscle mass and diet including but is not limited to, individuals who are amputees, paraplegics, or bodybuilders, or have obesity; patients who have a muscle-wasting disease or a neuromuscular disorder; and those suffering from malnutrition, eating a vegetarian or low-meat diet, or taking creatine dietary supplements.

Interpretation:

According to the Kidney Disease: Improving Global Outcomes (KDIGO) CKD work group, chronic kidney disease (CKD) is defined as the abnormalities of kidney structure or function, present for more than 3 months, with implications for health. KDIGO guidelines provide the following GFR categories:

Stage	Terms	eGFR mL/min/1.73 m^2
G1*	Normal or high	90
G2*	Mildly decreased	60 to 89
G3a	Mildly to moderately decreased	45 to 59
G3b	Moderately to severely decreased	30-44
G4	Severely decreased	15-29
G5	Kidney failure	<15

*In the absence of evidence of kidney damage, neither G1 nor G2 fulfill criteria for CKD.

Urea Serum	**10.70** ↓	mg/dL	16.6 - 48.5 Kinetic Urease/GLDH MC-7911

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IMMUNOLOGY			
VITAMIN B 12 Serum	320	pg/ml	197 - 771 ECLIA MC-7911

Note: Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath.
Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. The test results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Thyroid function tests

T3 Serum	1.41	ng/ml	0.80 - 2.00 ECLIA MC-7911
T4 Serum	10.33	ug/dl	5.1-14.1 ECLIA MC-7911
TSH Serum	4.430	uIU/ml	Adult : 0.270 - 5.350 Premature 28 - 36wk : 0.7 - 27.0 Cord Blood(>37 wk) : 2.3 - 13.2 Birth - 4 days : 1.0 - 39.0 2 - 20 weeks : 1.7 -9.1 5months - 20 years: 0.7 -6.4 ATA Guidelines for Healthy Pregnant Women First Trimester : 0.1 - 2.5 Second Trimester: 0.2 - 3.0 Third Trimester : 0.2 - 3.0. ECLIA MC-7911

Note: TSH has a diurnal rhythm, peaks at 2.00-4.00 am and has lowest level at 5.00-6.00 pm with ultradian variation. Hence thyroid test is only a snapshot of what is occurring within a dynamic system and for treatment purpose, the results should be accessed in conjugation with patient medical history, clinical examination & other tests/finding for confirmation. Many multivitamins (such as Vit B7), supplements (especially hair, skin, and nail) and over-the-counter and prescription medications may affect thyroid test results, and their use should be discussed with the healthcare practitioner prior to testing.
When a high serum TSH concentration and normal free T4 is found, repeat measurement 3-6 months later along with thyroid antibodies after excluding nonthyroidal illness and drug interference is recommended.

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END OF THE REPORT